

## Ch 1: Prerequisites & Safety

Before considering any anabolic steroid cycle, you must honestly assess whether you have met the minimum requirements. Starting too early risks permanent hormonal disruption, stunted natural development, and health consequences that far outweigh any aesthetic benefit.

### Minimum Requirements Before First Cycle

- Training age: minimum 3 years of consistent, structured weight training
- Age: minimum 25 years — HPTA is fully mature; safer for hormonal manipulation
- Natural potential: you have reached near-genetic maximum without compounds
- Body fat: below 15% (men) / 22% (women) — cycles are ineffective at high BF%
- Health: full blood panel showing normal hormones, liver, kidneys, lipids, CBC
- Knowledge: you fully understand PCT, AI use, injection technique, and side effects
- No personal or family history of: prostate cancer, cardiovascular disease, clotting disorders

## Ch 2: The Test-E Only Protocol

The first cycle should ALWAYS be testosterone-only. Testosterone Enanthate is the gold standard for beginners: long half-life means injections twice per week, well-established safety profile, easy to manage, and clearly establishes your individual response before adding more compounds.

| Week    | Test-E mg/week | Anastrozole | HCG IU/week | Notes                                            |
|---------|----------------|-------------|-------------|--------------------------------------------------|
|         | 250            | –           | 500 EOD     | Start HCG from day 1 for testicular preservation |
| 2       | 250            | 0.25mg EOD  | 500 EOD     | Monitor: water retention, acne, mood changes     |
|         | 250            | 0.25mg EOD  | 500 EOD     | Assess tolerance before ramping                  |
| 4       | 500            | 0.25mg EOD  | 500 EOD     | Move to working dose                             |
|         | 500            | 0.25mg E3D  | 500 EOD     | Main cycle phase; blood work at week 6           |
| 11      | 350            | 0.25mg E3D  | 500 EOD     | Begin taper down                                 |
|         | 200            | 0.25mg E3D  | 500 EOD     | Final injections                                 |
| 13–14   | –              | –           | –           | Clearance (10-day half-life ester)               |
|         | –              | –           | –           | Nolvadex 40mg + Clomid 100mg daily               |
| PCT 3–4 | –              | –           | –           | Nolvadex 20mg + Clomid 50mg daily                |

## Ch 3: Injection Technique

Proper injection technique prevents infection, oil embolism, and unnecessary pain. Every step of the sterility protocol must be followed without exception — contaminated injection equipment causes infections that can require hospitalisation.

| Site                    | Needle       | Volume       | Technique                         | Advantage                           |
|-------------------------|--------------|--------------|-----------------------------------|-------------------------------------|
|                         | 23G 1.5 inch | Up to 2 mL   | Z-track technique; aspirate 5 sec | Safest; fewest nerves and vessels   |
| Vastus lateralis (quad) | 25G 1 inch   | Up to 1.5 mL | Perpendicular; inject slowly      | Easy self-injection; visible site   |
|                         | 25G 1 inch   | Up to 1 mL   | 45° angle into lateral head       | Convenient; small muscle (1 mL max) |

### Sterility Protocol — Every Injection

- 1. Wash hands with soap for 20 seconds; dry thoroughly
- 2. Swab vial top with 70% isopropyl alcohol; allow 30 seconds to dry
- 3. Draw with 18G needle; switch to injection needle before injecting
- 4. Swab injection site; allow 30 seconds to dry
- 5. Inject slowly — 1 mL over minimum 10 seconds; do not rush
- 6. Rotate injection sites — never same site more than once per week
- 7. Dispose of needle and syringe in sharps container immediately

## Ch 4: Estrogen Management

Testosterone aromatises into estradiol (E2). The goal is optimal E2 — not zero E2. Both high AND low estrogen cause problems. Managing E2 is a skill that improves with experience; blood work is the most reliable guide.

| E2 Level               | Symptoms                                                       | Action                                             |
|------------------------|----------------------------------------------------------------|----------------------------------------------------|
|                        | Joint pain, brain fog, no libido, dry skin, depression         | Reduce or stop AI; retest in 1 week                |
| 15–35 pg/mL (optimal)  | Good mood, libido, strength, no water retention                | Maintain current AI dose                           |
|                        | Water retention, nipple sensitivity, mood swings               | Increase AI dose modestly; retest in 1 week        |
| > 60 pg/mL (very high) | Gynecomastia onset, extreme retention, emotional dysregulation | Add Nolvadex 20mg + increase AI; urgent blood work |

### Gynecomastia Emergency Protocol

If you feel a hard lump behind the nipple (not soft puffiness — a firm, moveable disc), this is gynecomastia. Act immediately:

- Stop: all aromatising compounds if possible; assess if dose adjustment is feasible
- Start: Nolvadex 40mg/day + Letrozole 2.5mg/day for 2 weeks
- After 2 weeks: Drop Letrozole; continue Nolvadex 20mg/day for 4 more weeks
- Blood work: E2 and prolactin; rule out prolactin-induced gyno (from Deca/Tren)
- If lump does not resolve in 3 months: surgical consultation (gyno can become permanent)

## Ch 5: Nutrition & Training

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Nutrition is 70% of the result on any cycle. Without a caloric surplus and adequate protein, testosterone will not build muscle — it simply cannot without the raw materials. This is the most underestimated factor by beginners.

| Goal          | Calories             | Protein         | Carbs       | Fat             |
|---------------|----------------------|-----------------|-------------|-----------------|
|               | TDEE + 300–500 kcal  | 2.0–2.2 g/kg BW | 3–4 g/kg BW | 0.8–1.0 g/kg BW |
| Recomposition | TDEE (maintenance)   | 2.2–2.5 g/kg BW | 2–3 g/kg BW | 0.7–0.9 g/kg BW |
|               | TDEE + 700–1000 kcal | 1.8–2.0 g/kg BW | 4–5 g/kg BW | 1.0–1.2 g/kg BW |

### Beginner Training Principles on Cycle

- Maintain training programme from pre-cycle — do NOT dramatically change everything
- Add 1 extra set per compound exercise (increased recovery capacity)
- Sleep 8–9 hours: anabolic hormones are mostly secreted during sleep
- Do not overtrain thinking compounds will compensate — they amplify smart training, not stupidity
- Track every workout: you should see consistent strength improvements by week 4–6

## Ch 6: Realistic Expectations & PCT

Realistic expectation-setting prevents post-cycle disappointment and helps you plan your cycle intelligently. Most of the gains seen on-cycle include water retention and glycogen supersaturation — actual dry muscle is a smaller but permanent component.

| Phase                  | Timeline         | What to Expect                                                              |
|------------------------|------------------|-----------------------------------------------------------------------------|
|                        | Setup phase      | Little visible change; compounds building in blood; some increased recovery |
| Weeks 4–6              | Activation phase | Noticeable strength increases; better pumps; improved mood and energy       |
|                        | Peak gains phase | Visible muscle fullness; scale weight rising 1–2 kg/week (includes water)   |
| Week 11–12             | Taper phase      | Maintain gains; begin mentally preparing for PCT                            |
|                        | Recovery phase   | Some weight loss (water); maintain training intensity to keep dry gains     |
| Post-PCT<br>1–3 months | Stabilisation    | Retain 50–70% of cycle gains with consistent training and nutrition         |

### Key Rules for First Cycle Success

- Have ALL ancillaries (AI, Nolvadex, Clomid) in hand BEFORE starting
- Run the full PCT — skipping PCT is the most common mistake and causes hypogonadism
- Blood work pre, mid (week 6), and post-PCT (week 4) — no exceptions
- Do not change your diet dramatically — gradual increases work best
- Minimum 1:1 time off to time on before your next cycle